MedAgent Clinical Decision Support

Case ID: case_46eb35d2d7 | **Status:** EDIT | **Physician:** Dr. Alex Vance | **Date:** 2026-10-12 17:32 UTC

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Patient Intake & Presentation

Patient: Age: 72, Sex: female
Notes: 72yo female with type 2 diabetes and hypertension presenting with progressive lethargy, nausea, oliguria for 24 hours. Currently taking lisinopril 20mg and spironolactone 25mg daily. Denies chest pain, denies hemoptysis, denies dyspnea.

Clinical Summary / Impression

Patient evaluation demonstrates clinical features consistent with Acute Kidney Injury. Diagnostic synthesis is supported by 5 key multimodal findings. Differential considerations include Severe Hyperkalemia, Metabolic Acidosis. Diagnostic workup and clinical monitoring are indicated to confirm etiology and guide definitive management.

Ranked Differential Diagnoses

#	Condition	Likelihood	Key Supporting Findings	Citations
1	Acute Kidney Injury	High	oliguria, lethargy, nausea, Renal Impairment, Hyperkalaemia	S_AcuteK_1, S_Acu
2	Severe Hyperkalemia	Moderate	Hyperkalaemia, Renal Impairment, Metabolic Acidosis	S1, S2
3	Metabolic Acidosis	Moderate	lethargy, Metabolic Acidosis, Renal Impairment	S6
4	Chronic Kidney Disease	Moderate	hypertension, diabetes, Renal Impairment, Hyperkalaemia	S2, S7
5	Hyponatremia	Moderate	lethargy, nausea, Hyponatraemia	S1

Laboratory Findings & Identified Patterns

Identified Clinical Patterns: Renal Impairment (urgent); **Hyperkalaemia** (urgent); **Hyponatraemia** (soon); **Metabolic Acidosis** (soon)

Physician Attestation: I, Dr. Dr. Alex Vance, hereby confirm that I have reviewed, verified, and adjusted this decision-support synthesis. Decision: **EDIT**.
Reviewer Notes: No specific notes recorded.
Signature: _____ **Date:** 2026-10-12